June 13, 2025

Dr. Mehmet Oz, MD
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Centers for Medicare and Medicaid Services
U.S. Department of Health and Human Services
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Dr. Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
Assistant Secretary for Technology Policy
U.S. Department of Health and Human Services
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Dear Dr. Mehmet Oz and Dr. Thomas Keane,

Thank you for inviting stakeholder input on the TEFCA program and the government's broader vision for interoperability. As healthcare providers and custodians of our patients' medical records, we make a promise to every patient, as required under Federal law, to protect their sensitive medical information. We also have a strong, vested interest in expanding interoperability so that medical information is available where and when needed to support high quality care.

The majority of interoperable healthcare organizations exchange information through data-sharing networks such as Carequality or eHealth Exchange. While these networks have significantly advanced interoperability, they also have structural limitations that hinder broader use and continued adoption. They have not scaled data exchange beyond the initial treatment use case, for example, and often lack robust oversight and enforcement mechanisms—raising concerns about patient privacy. TEFCA, as a federally endorsed trust framework, is the best opportunity to reach true nationwide interoperability. To realize this potential, HHS must ensure that TEFCA avoids the flaws that have caused data exchange through existing networks to plateau.

Most importantly, healthcare organizations need a strong voice in TEFCA governance. Patient trust in interoperability begins with the relationship between individuals and their healthcare providers. We collect protected health information, and we're responsible for informing patients about its use through Notices of Privacy Practices and related consents. The TEFCA Participant Caucus should include healthcare organizations and other HIPAA-covered entities that disclose data over the network rather than vendors, who might prioritize business concerns over privacy protections. The Participant Caucus should contribute to vetting and onboarding new participants, and must have final signoff on any new required response use cases, including expansion of the current TEFCA definition of treatment. Covered entities are accountable under HIPAA to prevent unauthorized access, so we need to help determine what policy guardrails are required before a disclosure.

To keep patient data safe, exchanges at the network-level should be audited, and all participants whether vendor or HIPAA-covered entity should be subject to routine oversight to ensure compliance and alignment with the public interest. Broader oversight and "policing" the network, including enforcement, should be managed by HHS as a neutral party. In terms of future legislation, we strongly recommend that all TEFCA participants be held accountable for protecting patient privacy under HIPAA and be subject to clear and enforceable oversight by HHS.

We are optimistic about TEFCA and look forward to working with the administration to expand interoperability in a way that empowers patients to access their own health data, protects sensitive data from unauthorized access, and removes barriers to data exchange so that any authorized organization can do so quickly and easily.

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